

A FOUNDERS' BRIEF

What happens next

Residential eldercare is the core of ilé, and home healthcare and virtual consultations are the businesses that carry us to it. The next steps for all three founders, with owners and dates, and a platform built in six weeks.

PREPARED FOR

Kemi Balogun

PREPARED BY

Debo Odulana, Founding Partner, Consult for Africa

DATE

14 September 2026

STATUS

Private and confidential. A working brief for the founders, not a legal or tax opinion.



ilé

A working brief for Kemi Balogun, covering the next steps for all three founders

14 September 2026. Private and confidential.

A letter from Debo

Kemi,

You asked what happens next. This document answers that, in detail, and separately for each of the three of us. Every action in it has an owner and a date.

Before the detail, one thing has changed, and it changes everything after it.

We are building a residential eldercare business. The home is the core. Home healthcare and virtual consultations are not the business with a care home attached at the end of it. They are the positions we take to carry us through the capital the home requires, and to fill it on the day it opens.

The plan we wrote in July had this the other way round. It was a reasonable plan and most of its content still stands. The sequencing does not. I would rather correct it now, in September, than discover the mistake in a signed founders' agreement.

I want to be plain about what leading with residential means, because it is the harder version of this business. It needs materially more capital, it needs a building and a licence before it earns a naira, and it asks Nigerian families to do something many of them still feel uneasy about. None of that makes it wrong. It makes it the version worth building, because it is also the version nobody in this market has built properly, and the version that owns the customer for the rest of their life rather than for a season.

It also puts real weight on the bridge. Home care and virtual consultations only weather the capital requirement if they are genuinely profitable businesses in their own right. Run as a marketing channel for the home, they will consume cash rather than supply it, and we will have taken the harder plan without the thing that makes it survivable. So we will hold those two lines to a positive contribution margin from the first month, and I will report on that monthly whether the news is good or not.

There is one item in this document that cannot wait a fortnight. Counsel is drafting now, against the July assumption of a single company. If the core is a building, the property and the operation have to be held separately, and that has to be settled before anything is signed. I set out why under "Separate the building from the business". That is a one page instruction from you this week, and it is the most valuable thing any of us will do in September.

What I am asking of you is three decisions and a capital ceiling, so that we can model this business rather than describe it. What we need from Chris is the acuity ceiling and the care standard, because between them they determine the building, the staffing and the price. From us, the platform, the model, the demand engine and the operation, starting now.

I have set out Chris's list in full below so that you can see it before he does. We will take it to him directly once you have read it. None of it reopens his terms.

We have spent eight weeks not moving. The waiting list should have been open for all of them. Let us start it this week.

Debo Odulana

Founding Partner, Consult for Africa

hello@consultforafrica.com



What has changed, and what has not

What has changed is the order, and therefore the destination. The business we are building is a residential eldercare home, then a second, then a small group of them. The care delivered in people's houses and the consultations delivered down a video link are how we get there with our balance sheet and our reputation intact.

This is not a cosmetic reordering. It changes the critical path, the capital plan, the corporate structure, the first hire, the shape of the building we look for, and what the technology has to do. Each of those is dealt with below.

What has not changed is most of the work. The service design, the pricing ladder, the diaspora thesis, the waiting-list engine and the cap table all survive, and several of them get stronger:

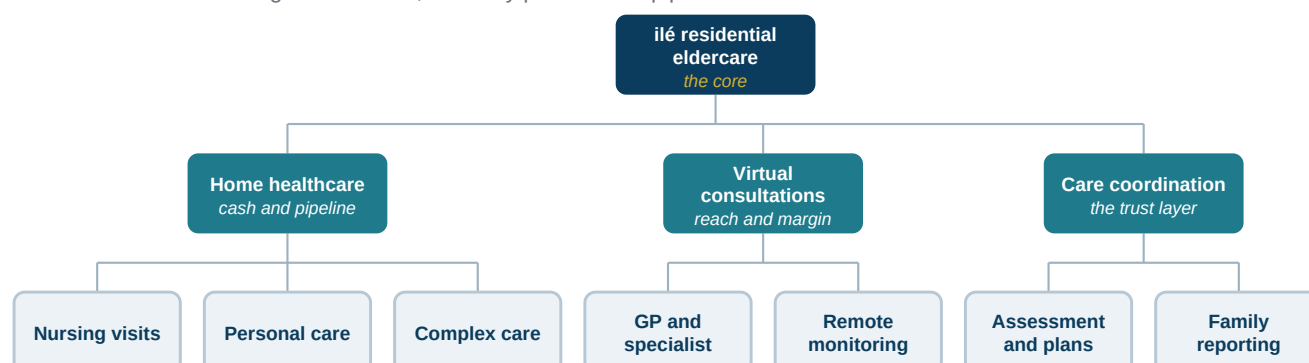
- **The diaspora thesis gets stronger, not weaker.** A daughter in London cannot personally care for her mother in Lagos. A trusted residential home is a better answer to her problem than a visiting carer she cannot supervise, it is worth more to her, and she is the segment least troubled by the cultural objection because she is already paying someone else to provide the care. Residential first sharpens the targeting rather than complicating it.
- **The waiting list becomes more valuable, not less.** For a home care business a waiting list is a pipeline of visits. For a residential home it is the occupancy plan, and occupancy is the whole economics of a care home. Every month the list is closed is a month of occupancy we are not building.
- **The cap table stands, with one structural correction.** The 51, 34, 10 and 5 split that you settled in July is right for the operating business. It should not extend to the property. That is the correction, and it is the subject of the first decision below.

What is now wrong in the July documents. The rollout plan puts residential in phase three as ground to be prepared. Treat that document as correct on service design, pricing, the diaspora channel and the waiting-list strategy, and superseded by this one on sequence, phasing and the critical path. The venture note and the terms sheet for Chris stand as written.

The shape of the business

FIGURE

The residential home is the core and the destination. The three lines beneath it are real businesses that must carry themselves, and they exist for three further reasons: they generate cash while the building is found and fitted, they prove the care standard and the brand before a single bed is sold, and they produce the pipeline of families whose needs escalate into the home.



The logic that ties them together is worth stating in one sentence, because every decision in this document follows from it.



A residential eldercare home lives or dies on occupancy and on the acuity it can safely carry. The hardest thing about opening one is filling it with the right residents at the right price on the day it opens. A home care book, properly run, is the only asset that solves that problem in advance, because it puts us inside the homes of hundreds of families at exactly the moment their situation is deteriorating.

That is why the bridge businesses are not a detour. They are how we de-risk the core.

The critical path

FIGURE

Four phases. The long pole is the building, not the software and not the clinical work, so the site search starts this month and everything else is arranged around it.



Three things gate everything else, and they run in parallel rather than in sequence:

1. **The building.** Finding it, securing it, licensing it and fitting it is the longest single item on the path and the one with the least certain duration. It starts now.
2. **The money.** The capital envelope determines the size of the home, and the size of the home determines the model. We cannot build a credible financial model until you set a ceiling.
3. **The clinical ceiling.** What we will and will not accept clinically determines staffing ratios, the building specification and the insurance, and therefore the cost. Chris sets it.

Everything else, the platform, the waiting list, the recruitment, the brand, is work we control and can start immediately. We are starting it immediately.

Three decisions we take together this month

These are the three that cannot be delegated to any one of us, and all three are due before the end of September.

One: separate the building from the business

The decision. Hold the property in one company and the care operation in another, rather than in a single ilé company.

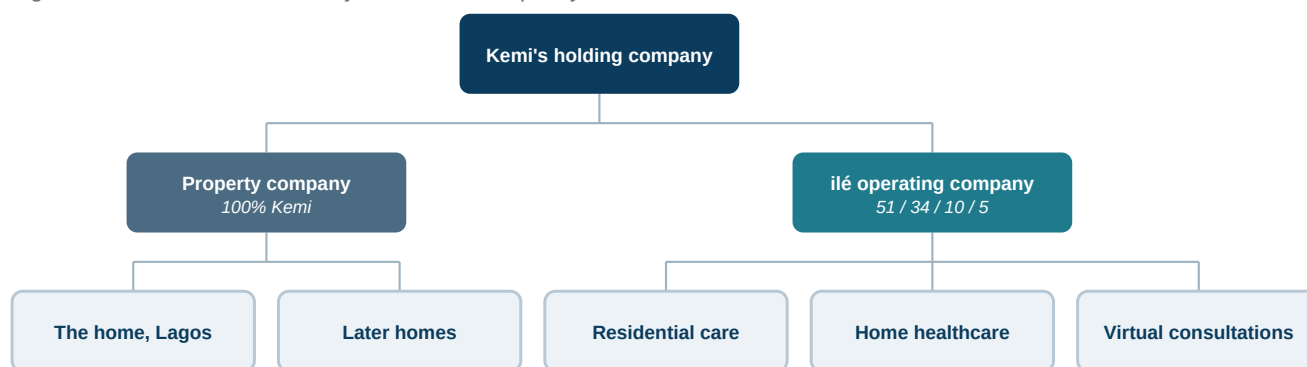
Why this matters more than it sounds. If one company owns both the building and the operation, then Consult for Africa's 34% is 34% of a building you paid for. That is not what we earned and it is not what you intended to give. We earned our stake by building the technology and running the operation. We should not be paid in a third of your real estate, and you should not be asked to hand it over. The moment the core became a building, the single-company cap table stopped expressing the deal the three of us actually made.

The structure that fixes it. A property company, held by you through your holding company, owns or leases the site. ilé, the operating company, carries the 51, 34, 10 and 5 cap table exactly as you settled it, holds the brand, the platform, the clinical licences and the staff, and pays the property company rent on arm's length terms.



FIGURE

The property sits with your capital. The operating business carries the agreed cap table. Rent passes between them at an arm's length rate that the home can carry at realistic occupancy.



What it also buys us.

- **Clinical liability sits in the operating company, away from the asset.** In a business caring for frail people this is not a technicality.
- **The property can be financed as property.** Real estate attracts different lenders, different tenors and different pricing from an unproven care operator. Blending them makes both harder to fund.
- **The second home is a repeat of the same pattern** rather than a renegotiation of everybody's shareholding.
- **The two things are worth different multiples.** Care operators are valued on operating earnings, property on yield. Held together, one of them is always being undervalued.

The honest costs, which counsel must handle. The rent is a related party transaction and has to be set at arm's length and documented, and transfer pricing scrutiny is real under the 2025 reform. Set the rent at a level the home can carry at realistic occupancy rather than at the theoretical maximum, because rent is the fixed cost that breaks care homes in a bad year. Group reorganisation relief from capital gains tax is already on the open questions list in the counsel brief and now matters more.

Disclosure. Consult for Africa is an interested party in this decision, as we were in July. It is in our interest to hold a clean stake in an operating company. Please put this to your own legal and tax advisers rather than taking it from me.

Two: set the site parameters, so the model can be built

We cannot model a home we have not sized. Five parameters, one conversation, ninety minutes:



Parameter	The choice	Why it decides the model
Metro and area	Lagos, and which corridors	Decides land cost, staff travel, the families within reach and which hospitals can receive an emergency
Bed count band	A small first home, or a larger one	Small fills faster and proves the model. Larger carries its overhead better but is far harder to fill and far more capital
Acuity ceiling	Set by Chris, below	Decides staffing ratios, building specification, insurance and price
Lease, convert or build	Three very different capital profiles	Conversion is usually fastest to revenue and lightest on capital. Building gives the right home but is slowest and heaviest
Payer mix	Private pay only, or private pay plus corporate and diaspora contracts	Decides the sales motion and how quickly occupancy ramps

Our recommendation is to convert rather than build. Converting an appropriate existing building gets us to revenue far sooner, at materially lower peak funding, and lets the first home prove the model before we commit to purpose built. It constrains the clinical design, which is a genuine cost, and Chris will have to rule some buildings out. That is a trade worth making for the first home and probably not for the third. This is the kind of conversion work we have done before and we will do the building assessments ourselves.

Three: choose the funding route

The question. How much of this is family capital, and what carries the rest.

We are not asking for the whole answer this month. We are asking for two numbers and one direction: the capital you are prepared to commit, the ceiling beyond which the answer is no, and whether outside money is acceptable in principle and in what form. Equity in the property company, equity in the operating company, debt against the building, and a property partner who contributes the site for a share are four very different answers with very different consequences for control.

Until we have the ceiling we can describe this business but we cannot cost it, and every other number we produce is decoration.

Kemi: your next steps

Your lane is capital, property, the group structure and control. You have final say in that lane.

This week, 14 to 18 September

- **Redirect counsel, in writing, in one page.** Tell them the core of the healthcare venture is a residential home, that we intend to separate the property from the operating company, and that they should pause drafting the single company cap table until the structure is settled. Redirecting a draft costs a letter. Re-papering a signed agreement costs a great deal more.
- **Confirm the citizenship and holding vehicle of every shareholder, and tell counsel.** If any shareholder takes shares through a non-Nigerian entity, the company becomes one with foreign participation and needs one hundred million naira of issued share capital, fully paid before a business permit or expatriate quota. Citizenship, not residence, is the test, so a Nigerian citizen living abroad who holds personally, or through a Nigerian company, does not trigger it. Ask the question before the shares are issued, not after.



- **Put the ninety minute site parameters conversation in the diary** for the week of 21 September, the three of us.

Weeks of 21 September to 9 October

- **Set the capital envelope and the ceiling.** Two numbers. Without them we cannot model.
- **Decide lease, convert or build, and name two or three target corridors in Lagos.** We will assess every building you shortlist.
- **Instruct counsel on both sets of papers:** the property and operating company structure with the rent arrangement, and the ilé founders' agreement with the reserved matters list.
- **Settle the two dilution points you were warned about in July,** in the founders' agreement: that the employee pool and Chris's top-up shares are drawn from the whole cap table proportionally, so your 51% is not eroded by accident, and that you hold a pre-emption right so you can always follow your money in a raise.
- **Take independent legal and tax advice on the Consult for Africa terms.** We are an interested party and you should not take our word for our own arrangement.

By the end of October

- **A preferred site, walked, with heads of terms opened.**
- **The founders' agreement signed and the operating company incorporated, with shares issued at incorporation at nominal value.** Shares issued for services are taxed on the value received when they are issued or vest. Issuing at formation, when the company is worth almost nothing, keeps that taxable amount close to nil for both Chris and for us. Every month of delay makes this more expensive, and it is now the second most time sensitive item after counsel.
- **The funding route decided,** and first conversations opened if debt or a property partner is part of the answer.

Dr Anele: what we need from Chris, and when

Chris's lane is clinical. Standards, safety, protocols and who ilé is willing to care for. He has final say in that lane, and nothing clinical ships without his name on it.

His role grows considerably under this plan. A home care service needs good protocols. A residential home needs a clinical governance structure that will stand up to a regulator, an insurer and a coroner.

This is the list we propose to put to him, set out here so that you see it before he does. Tell us if you want anything changed, added or held back, and we will take it to him this week. None of it reopens his terms.

This week, 14 to 18 September

- **His acceptance of the terms, in writing,** so counsel can paper them alongside everything else. He has agreed in principle, and the one page he holds is the basis. He should take his own independent legal and tax advice on it first.
- **Where his medical director's certificate currently sits.** In Lagos, a medical director's certificate may be used for one facility at a time. If his is attached to his present hospital, he cannot also be the named medical director of record for the ilé home, and we will need to recruit that person rather than assume him. This is a two week question, and the answer decides your first senior clinical hire, so it is worth settling now rather than in March.

By 2 October, because the build now depends on it



We are building the platform in six weeks rather than twelve. That compression is ours to absorb except in one place: the platform is built around Chris's clinical thinking, so two items that were October work are now early October deadlines.

- **The acuity ceiling.** The single most consequential clinical decision in the business, and everything else costs what it costs because of it. For each of the following he should tell us yes, no, or yes with conditions: advanced dementia with wandering; post stroke with tube feeding; tracheostomy or oxygen dependence; unhealed pressure wounds; challenging behaviour requiring one to one supervision; palliative and end of life care on site. Each yes changes the staffing ratio, the building, the insurance and the price. Each no narrows the market. There is no right answer, only an answer we can staff and defend.
- **The assessment tool and the care plan structure.** This is the hard deadline in the whole clinical list. The assessment produces the acuity score, and the acuity score drives the staffing, the price and the admission decision, so it sits at the centre of the software rather than at the edge of it. If it arrives at the end of October we will ship placeholders and rework them, and that rework costs a week we no longer have.
- **The ilé care standard, on one page, in plain language.** Not a policy document. The promise a family reads and decides to trust. It is the thing they are actually buying, and the front door needs it.

Through the rest of October

- **The policy set the home cannot open without,** named by him so that we can start writing them with him: medication management, falls, infection prevention and control, safeguarding of vulnerable adults, deterioration and hospital escalation, end of life, restraint, and capacity and consent.
- **A geriatrician or family physician identified as medical director of record.** The 5% clinical pool in the cap table exists for precisely this hire, which is fortunate, because we need it earlier than we thought.
- **Two hospitals named in the target corridors that will receive residents in an emergency,** and the conversation opened with them. A residential home without a named receiving hospital is not a safe home and should not open.
- **The staffing model by acuity tier:** who is on shift, at what ratio, overnight and at weekends. This is the largest operating cost in the business and it flows directly from the acuity ceiling.
- **His requirements for the building, written down before you sign anything.** A care home floor plan is a clinical document, not an architectural one. Corridor and door widths for a hoist and a wheelchair, sightlines from the nursing station, bathroom transfer space, where the assisted bathroom sits, single storey or lift, and the outside space that residents can actually reach. With that list we can rule buildings in and out for you. Without it we will fall in love with a building that cannot be licensed.

Consult for Africa: our next steps

Our lane is the platform, the model, the demand engine, the brand and the operation.

This week, 14 to 18 September

- **Open the waiting list.** The front door and segmented capture go live this week, before anything else we build. Eight weeks of signups have already been lost and the list is the occupancy plan.
- **Position the brand residential first, while selling home care today.** The promise is a place and a standard of care for someone's parent. The service available this month is care in their own home. Both are true and the messaging carries them together rather than pretending the home already exists.
- **Begin the platform build.** Set out in full in the next section.



- **Write the structure note for counsel** on the property and operating split, so your instruction has something technical behind it.

Weeks of 21 September to 9 October

- **Build the financial model, once the site parameters land.** Occupancy driven for the home, with the two bridge businesses modelled separately so that everyone can see exactly what they are carrying and when the home stops depending on them. The model will give peak funding, the funding trough and the month it turns, which is what the funding decision actually needs.
- **Assess every shortlisted building** against Chris's clinical requirements and against conversion cost, and give you a written comparison rather than an opinion.
- **Run the waiting list properly.** Segmentation on capture by local or diaspora, care need, urgency and location. Founding family status for early joiners. A referral loop. Genuinely useful content for people caring for an ageing parent, sent while they wait.
- **Recruit and vet the first care cohort**, and begin training them on Chris's care standard as soon as it exists.
- **Register with the Nigeria Data Protection Commission, appoint a data protection officer and publish the privacy notice, before the first family record is created.** Health data is sensitive personal data under the Nigeria Data Protection Act 2023. We will not retrofit this.

By the end of October

- **Platform version one live on 23 October**, carrying the whole home care loop: signed, assessed, scheduled, delivered, recorded, billed.
- The waiting list at a first meaningful number, with the diaspora share visible and a ranked shortlist forming.
- The financial model delivered, with peak funding and the trough month.
- The first care cohort recruited and in training.
- A written comparison of the shortlisted buildings.

The platform we are building

We start the build this week and version one is live in six weeks. This section is here so that you know what is coming, what we need from you and from Chris to hit that, and what six weeks costs in scope. There should be no argument later about what version one was supposed to do.

What it must do

Six things, and nothing else in version one.

1. **The family record.** One record per client, which follows them from first enquiry, through home visits, into a residential admission, without ever being re-keyed. This is the single most important design decision in the system and it is why we are building rather than buying.
2. **Assessment and care plan.** Chris's assessment tool, digitised, producing a care plan and an acuity score that drives everything downstream: the staffing ratio, the price and the admission decision.
3. **Scheduling and rostering.** Visits for the home care book today, shifts and rotas for the home when it opens. One engine, two shapes.
4. **The carer application.** What the carer actually uses at the bedside. Check in, the tasks from the care plan, observations, medication administration, incident reporting, check out. It has to work with no signal and sync when the signal returns, because Nigerian mobile networks will not be reliable inside a client's



house and a carer who cannot record care will stop recording care.

5. **The family dashboard.** What happened, when, and by whom, with photographs where the family has consented, and a monthly written report. For the diaspora this is not a feature of the product. It is the product.
6. **Billing and payments.** Naira collections for local families, foreign currency for diaspora subscriptions, handling both subscriptions and pay per visit.

Built residential ready from the first line

This is the part that changes because residential is the core, and it costs us almost nothing now and a rewrite later.

- **We model the client, not the service.** A client has episodes of care. A home visit and a residential stay are both episodes against the same person, with the same clinical history, the same family and the same billing relationship.
- **Occupancy goes into the data model now, even though the screens come later.** Unit, room, bed, admission, discharge. Building the modules later is cheap. Discovering that the schema cannot express a bed is not.
- **Every home care client carries escalation signals.** Acuity trend over time, carer strain, falls, hospital admissions, whether the family decision maker lives abroad, and whether the current arrangement is holding. This is the most valuable thing the bridge businesses will produce, because it is the admissions pipeline for the home, and it only exists if we design for it from the start. In eighteen months this dataset should be able to tell us, family by family, who will need a bed and roughly when.
- **Every clinical write is audited.** Who wrote what, when, and what it said before. A home that cannot demonstrate that is not defensible to a regulator, an insurer or a family.
- **Money is held as decimals, never as floating point,** and consent, lawful basis and retention are recorded per record under the Nigeria Data Protection Act 2023.

The build order: six weeks, not twelve

Six weeks, with something useful live in the first one. The dates below are real dates, not estimates of effort.

By	What is live	What it lets the business do
Fri 18 Sep, week 1	Front door, waiting list capture with segmentation, admin view of the list	Start building the occupancy plan on Monday rather than in November
Fri 2 Oct, week 3	Identity and roles, the client record, assessment and care plan	Take on a client and know what care they need
Fri 9 Oct, week 4	Visit scheduling, the carer application with an offline write queue, visit logging and medication administration	Deliver care safely and have a defensible record that it happened
Fri 23 Oct, week 6	Family view and the monthly report, consent handling, naira collection for subscriptions and single visits	Show the family what we did and get paid for it
After the site is secured	Residential modules: occupancy, admissions, bed management, rota by unit	Open the home on the same platform, with the same client records

Read the middle two rows as the answer to how fast this can be. At four weeks we can sign, assess, schedule, deliver and record care. At six weeks the family can see it and we can bill for it. That is the whole



home care loop, closed, before the end of October.

What six weeks costs, and what it does not

Compression is only honest if it names what was cut, so here is the list.

- **Virtual consultations come out of version one.** They were ten to twelve week work. Until the platform carries them, we run consultations on an ordinary video tool and write the note into the client record by hand. The families will not notice. We will.
- **Rotas come out.** Home care needs visit scheduling. Rotas are a residential need, and they ship with the residential modules when there is a building to roster.
- **Foreign currency collection starts manual.** Diaspora families are invoiced and paid in sterling or dollars outside the platform until the volume justifies automating it. The subscription itself is tracked in the platform from day one, so nothing is lost.
- **The family view is a view, not a portal.** Visits, observations, photographs where consented, and a monthly report. No messaging, no document vault, no login for extended family. Those come when families ask for them.
- **The carer application queues writes offline, and only writes.** Check in, tasks, observations, medication administration, incident, check out. Full offline reading of the whole record is a much larger piece of work and it is not what a carer at a bedside actually needs.

What is not cut. The residential ready data model stays exactly as described above, because it costs days now and a rewrite later. The audit trail on clinical writes stays. The data protection work stays and happens before the first family record exists, not after. None of those three are candidates for compression, and I would rather move the date than drop them.

Still out of scope entirely: clinical decision support, insurance and health plan claims, a public marketplace of carers, native mobile applications, and anything that draws attention to how clever it is. A care platform earns trust by being boring and reliable.

The one thing that can break the six weeks

Chris's assessment tool and care plan structure by 2 October. The assessment produces the acuity score, the acuity score drives the staffing, the price and the admission decision, and the software is built around it rather than beside it. If it lands at the end of October we ship placeholders and rework them, and the rework is the week that turns six into eight.

From you, the site parameters by the end of September, for the same reason in a different place: the residential data model follows the shape of the home.

How the three of us will work

Eight weeks passed since July without anything moving. That was a process failure rather than a disagreement, and the fix is a rhythm rather than better intentions.

- **A founders' call every week, forty five minutes, same time.** Three standing items: decisions due this week, what is blocked and on whom, and the numbers.
- **One decision log, and if it is not written in it, it was not decided.** We maintain it and circulate it after every call.
- **Clear lanes and real authority in them.** You decide capital, property and group structure. Chris decides clinical. We decide build and operations. Anything that crosses a lane is decided by the three of us, and there is a short written list of matters reserved to all three, which goes into the founders' agreement.



- **A written monthly update from us** covering the waiting list, the contribution margin on the bridge businesses, the site, the build and the cash. It will include the numbers that are going badly. That is the point of it.

What could stop us

- **The building takes longer than the money lasts.** The reason the bridge businesses have to be profitable rather than promotional. We will report their contribution margin monthly from the first month, and if they are not standing up on their own we will say so early rather than late.
- **The home opens and does not fill.** The single biggest risk in residential eldercare anywhere in the world. Our answer is the waiting list and the escalation dataset, which is why both start now and not when the building is ready.
- **The cultural objection.** Some Nigerian families will hear "care home" and hear abandonment. This is real and we should not pretend otherwise. Our answer is to lead with the diaspora, who have already accepted that someone else must provide the care, to sell safety, dignity and relief rather than convenience, and to build a place that a visiting family is proud of rather than embarrassed by. The building and the language both do that work.
- **A serious clinical incident in year one.** Would set the business back further than any commercial mistake. The acuity ceiling, the policy set, the named receiving hospital, the audit trail and proper insurance are the answer, and they are all in the October work above.
- **Regulatory ambiguity.** Residential eldercare does not sit inside a settled, dedicated licensing regime in Nigeria the way it does in the United Kingdom. That cuts both ways. It gives us room to set the standard, and it leaves us exposed to a regime arriving later. Our approach is to register under what does apply, ask the relevant authorities in writing rather than assume, and build above the standard we expect to be held to eventually. A business that is already compliant when the rules arrive is advantaged by them.
- **The three of us drifting apart on what this is.** The rhythm above, the decision log and the reserved matters list exist for that reason.

The first ninety days, in one table



Weeks	Kemi	Dr Anele	Consult for Africa
14 to 18 Sep	Redirect counsel in writing. Confirm shareholder citizenship and vehicles. Book the parameters call	Terms confirmed in writing. Establish where his medical director's certificate sits	Waiting list live Fri 18 Sep. Platform build starts. Structure note for counsel
21 Sep to 2 Oct	Capital envelope and ceiling. Lease, convert or build. Target corridors. Instruct counsel on both sets of papers. Independent advice on our terms	Hard date, 2 Oct: acuity ceiling, assessment tool, care plan structure, care standard. The build waits on these	2 Oct: client record, assessment and care plan live. Financial model. Building assessments. Data protection registration
5 Oct to 23 Oct	Preferred site walked, heads of terms opened. Founders' agreement signed, company incorporated, shares issued at nominal value. Funding route decided	Policy set. Medical director of record candidate. Two receiving hospitals. Staffing model. Written building requirements	9 Oct: scheduling and the carer app. 23 Oct: family view and billing, version one complete. First cohort recruited and in training
Nov to mid Dec	Site secured. Funding executed or committed	Protocols into the platform. Clinical sign off on the building	Home care operating with real clients and real revenue. Waiting list converting. Virtual consultations brought into the platform

If you read one page

ilé is a residential eldercare business. The home is the core and the destination. Home healthcare and virtual consultations are real businesses that must pay their own way, and they exist to carry us financially until the home opens and to fill it when it does.

Three things are on the critical path and all three start this month: the building, the capital ceiling and the clinical acuity ceiling. Everything else is work we control and it has already started.

One decision is urgent because counsel is drafting against the old assumption. The property and the operating business must be held separately. Your capital owns the building. The operating company carries the agreed shareholding, holds the brand, the platform and the licences, and pays rent. That protects you from giving away a third of an asset you funded, protects the asset from clinical liability, and lets the property and the operation be financed as the different things they are.

The waiting list opens this Friday. It is the occupancy plan for the home, and it should have been open since July.

The platform is built in six weeks, not twelve. Version one is live on 23 October and it closes the whole home care loop. The only thing that can break that date is Chris's assessment tool arriving late, which is why it is the one hard date in his list.

Debo Odulana

Founding Partner, Consult for Africa

Lagos and Abuja

hello@consultforafrica.com